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The Triage & Treat Toolkit: Your Decisive Guide to GI Symptoms

When to treat, investigate, and refer

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The plan for this session

1. Assess the patient	Emergencies and alarm features
2. Treat in primary care	Case 1: uncomplicated reflux
3. Investigate	Case 2: persistent watery diarrhea
4. Refer	Case 3: iron-deficiency anemia
5. Arrange follow-up	Results, referrals and reassessment

Additional dyspepsia and H. pylori guidance follows the main talk in the appendix.

Infographic

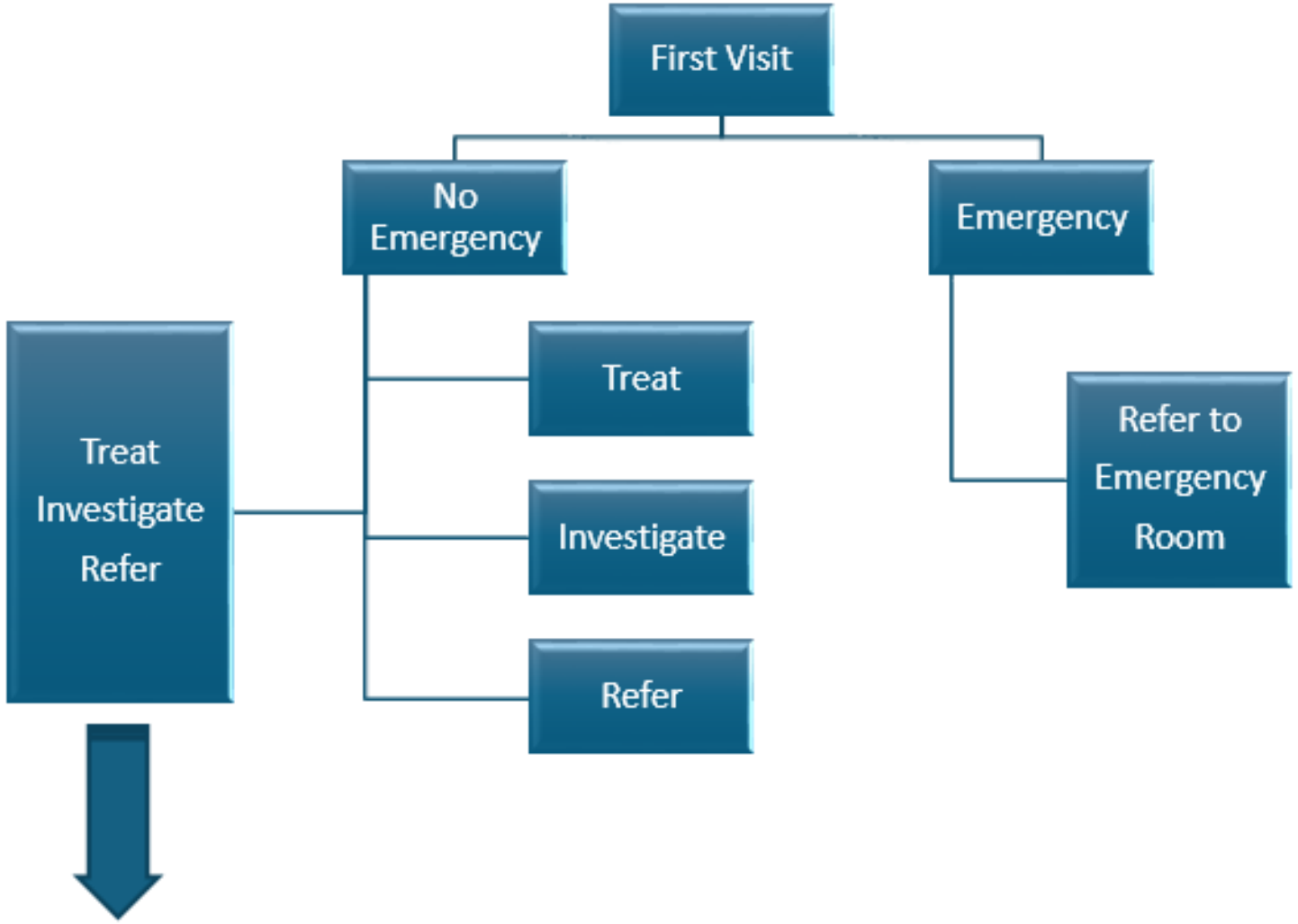


Three decisions at the first visit

TREAT	A likely benign diagnosis, no alarm features, and a defined treatment trial.
INVESTIGATE	A stable patient whose symptoms need targeted tests to clarify the cause.
REFER	Alarm features, likely organic disease, or persistent symptoms despite appropriate care.

Treatment, testing and referral can occur together.
An emergency bypasses the outpatient pathway.

Three decisions at the first visit



Treatment, testing and referral can occur together.

Initial assessment

Does this patient need immediate or expedited care?

Assessment, emergency example and alarm features

Initial assessment

History	Onset, duration, severity, stool pattern, bleeding, swallowing, vomiting and weight change.
Context	Medications, travel, family history, previous tests, cancer risk and impact on daily life.
Examination	Vitals, hydration, pallor, abdominal findings, jaundice and rectal examination when indicated.

A medication review often changes the plan: NSAIDs, anticoagulants, metformin, opioids and supplements.

NICE CG61: IBS in adults
ACG GERD guideline (2022)

Emergency assessment comes first

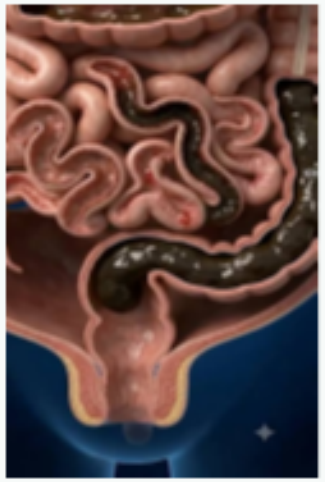


Bleeding with instability	Haematemesis or melena with syncope, hypotension, tachycardia or poor perfusion.
Acute abdominal illness	Peritonism, severe escalating pain, obstruction features or severe dehydration.
Other immediate risks	Food impaction with inability to swallow saliva, or jaundice with fever and systemic illness.

Arrange immediate hospital assessment. Stabilize within available resources without delaying transfer.

NICE CG141: acute upper GI bleeding

Emergency example: melena and syncope



A 70-year-old taking an NSAID develops black tarry stools and fainting.

BP 88/54 mmHg, pulse 118/min, pale and clammy.

This patient needs emergency hospital transfer and resuscitation.

Do not wait for outpatient blood tests or a routine endoscopy appointment.



Hospital endoscopy follows initial resuscitation, urgently in an unstable severe bleed.

NICE CG141: acute upper GI bleeding

Stable patient but with Alarm Features



- Progressive dysphagia or painful swallowing
- Unexplained GI bleeding or iron-deficiency anemia
- Unintentional weight loss, palpable mass or persistent vomiting
- Persistent nocturnal diarrhea or inflammatory features
- New symptoms with substantial age-related or family cancer risk

An alarm feature changes the pathway. Its urgency depends on the symptom and clinical context.

ACG/CAG dyspepsia guideline (2017)

NICE NG12: suspected cancer referral

Stable patient but with Alarm Features





STEP 1 ∘ INITIAL ASSESSMENT & EMERGENCIES

Adult gastrointestinal symptoms in primary care



FIRST: rapid assessment

Airway, breathing, circulation • Vitals and mental status • Hydration and perfusion

Once stable: symptom onset and severity, bleeding, swallowing, vomiting, stool pattern and weight change. Review medicines, family history and previous tests. Examine the abdomen.



1 EMERGENCY FEATURES
Immediate hospital assessment

- GI bleeding with syncope, hypotension or poor perfusion
- Severe or escalating pain, guarding or rigidity
- Distension with vomiting and failure to pass stool or gas
- Food impaction: unable to swallow saliva
- Jaundice with fever or systemic illness
- Severe dehydration or persistent inability to retain fluids

Stabilize within available resources. Do not delay transfer for outpatient tests.

Suspected acute GI bleeding needs same-day assessment even if initially stable.

2 STABLE, BUT ALARM FEATURES
Prompt investigation / expedited referral

- Progressive dysphagia or painful swallowing
- Unexplained iron-deficiency anemia
- Unintentional weight loss or palpable mass
- Persistent vomiting or nocturnal diarrhea
- Persistent rectal bleeding
- New symptoms with significant cancer risk

Choose urgency by severity and the local pathway. Use a suspected-cancer pathway when indicated.

3 STABLE, NO ALARM FEATURES
Continue primary care assessment

- Define the likely diagnosis
- Choose targeted tests when needed
- Consider a time-limited treatment trial
- Set a review date and track results
- Explain when to seek urgent reassessment

No alarm features does not exclude serious disease. Reassess persistent or changing symptoms.



EMERGENCY CASE

70-year-old taking an NSAID: melena + syncope, BP 88/54 mmHg, pulse 118/min.

Decision: emergency transfer and resuscitation.

Do not attribute acute chest pain to reflux before considering cardiac causes.



Teaching synthesis: NICE CG141 and NG12; ASGE food impaction guidance. Follow local emergency and referral protocols.

When to treat

Can this patient start treatment in primary care?

Case 1: reflux presentation, treatment and review

Case 1: heartburn after meals



What is the most appropriate next step?

A 34-year-old has 6 weeks of heartburn and acid regurgitation, worse after late meals.

No dysphagia, weight loss, vomiting, bleeding or exertional chest pain.

Examination is normal. No major Barrett's esophagus risk profile.



Treat now, order endoscopy, or refer?

ACG GERD guideline (2022)

Case 1: treatment in primary care

Treatment	A standard-dose PPI once daily, 30–60 minutes before breakfast, for 8 weeks.
Lifestyle	Avoid meals within 2–3 hours of bedtime. Address weight if appropriate and individual triggers.
Review	Assess adherence and response. If controlled, attempt withdrawal or step down when appropriate.

New dysphagia or bleeding changes the plan immediately.

ACG GERD guideline (2022)

Other presentations suitable for initial treatment

Constipation without alarms	Review causes, offer gradual soluble fiber and adequate fluids. Polyethylene glycol is an evidence-based option.
Established IBS without alarms	Explain the diagnosis. Match diet and symptom treatment to the bowel pattern.
A clear follow-up plan	Record the target symptom, review date and reason to escalate.

An empiric treatment trial needs a stop point and reassessment.

AGA/ACG constipation guideline (2023)

AGA refractory constipation update (2026)

NICE CG61: IBS in adults

STEP 2 • WHEN TO TREAT

Reflux case: initial management in primary care



CASE 1 • A 34-year-old with heartburn

6 weeks of heartburn and acid regurgitation, worse after late meals.
No dysphagia, weight loss, vomiting or bleeding. Normal examination.
No concerning chest-pain features or major Barrett's risk profile.

1 WHY TREAT?

- Typical reflux symptoms
- No alarm features
- Likely uncomplicated GERD
- Initial empiric treatment is appropriate.
- Routine endoscopy is not required at the first visit in this low-risk case.

GERD = gastroesophageal reflux disease

2 INITIAL TREATMENT



8-week PPI trial

Standard dose once daily, 30–60 minutes before breakfast.
Example: omeprazole 20 mg daily, if appropriate.

Practical lifestyle measures

- Avoid meals within 2–3 hours of bedtime.
- Weight reduction if overweight.
- Elevate the head of the bed for nocturnal symptoms.
- Limit foods that clearly trigger symptoms.

PPI = proton pump inhibitor

3 REVIEW AT 8 WEEKS

Symptoms controlled

Attempt PPI withdrawal when there is no established maintenance indication.

Symptoms persist or recur

Check adherence and dose timing.
Reconsider the diagnosis.
Arrange further evaluation, often endoscopy, for inadequate response or recurrence after withdrawal.

Document a review date and explain when to return earlier.

CHANGE THE PLAN IF ALARM FEATURES APPEAR

New dysphagia, GI bleeding, unexplained weight loss, anemia or persistent vomiting require prompt assessment.
Bleeding with instability or concerning acute chest pain requires emergency assessment.

Teaching case adapted from the presentation. Source: ACG GERD guideline, 2022.

Individualize treatment. Established severe erosive esophagitis or Barrett's esophagus may require ongoing PPI therapy.

When to investigate

Which tests will clarify the cause and change management?

Case 2: diarrhea presentation, tests and next steps

Investigation should answer a clinical question



Epigastric pain	Could this be H. pylori-associated dyspepsia or an indication for endoscopy?
Persistent diarrhea	Is there inflammation, celiac disease, infection or another treatable cause?
Anemia or bleeding	Is there a source that requires endoscopic evaluation?

Order tests that change management. Complete broad testing should not be a condition for referral.

ACG/CAG dyspepsia guideline (2017)

AGA chronic watery diarrhea guideline (2019)

AGA iron-deficiency anemia guideline (2020)

Case 2: persistent watery diarrhea



Which tests would change management?

A 29-year-old has 8 weeks of watery stools, 4–5 times daily, with bloating.

No blood, weight loss, fever or nocturnal symptoms. Vitals and examination are normal.

No recent antibiotics, high-risk travel or family history of IBD, celiac disease or colorectal cancer.

Stable does not mean "no investigation needed."

AGA chronic watery diarrhea guideline (2019)

Case 2: a targeted initial work-up



Blood tests	CBC and chemistry as clinically indicated. Tissue transglutaminase IgA with total IgA.
Stool tests	Fecal calprotectin or lactoferrin, plus Giardia testing. Add other infection tests when indicated.
Interpretation	Positive celiac serology or inflammatory findings warrant specialist evaluation.

Keep gluten in the diet during celiac testing. IgA deficiency needs an appropriate IgG-based test.

AGA chronic watery diarrhea guideline (2019)

Case 2: how the results change the pathway

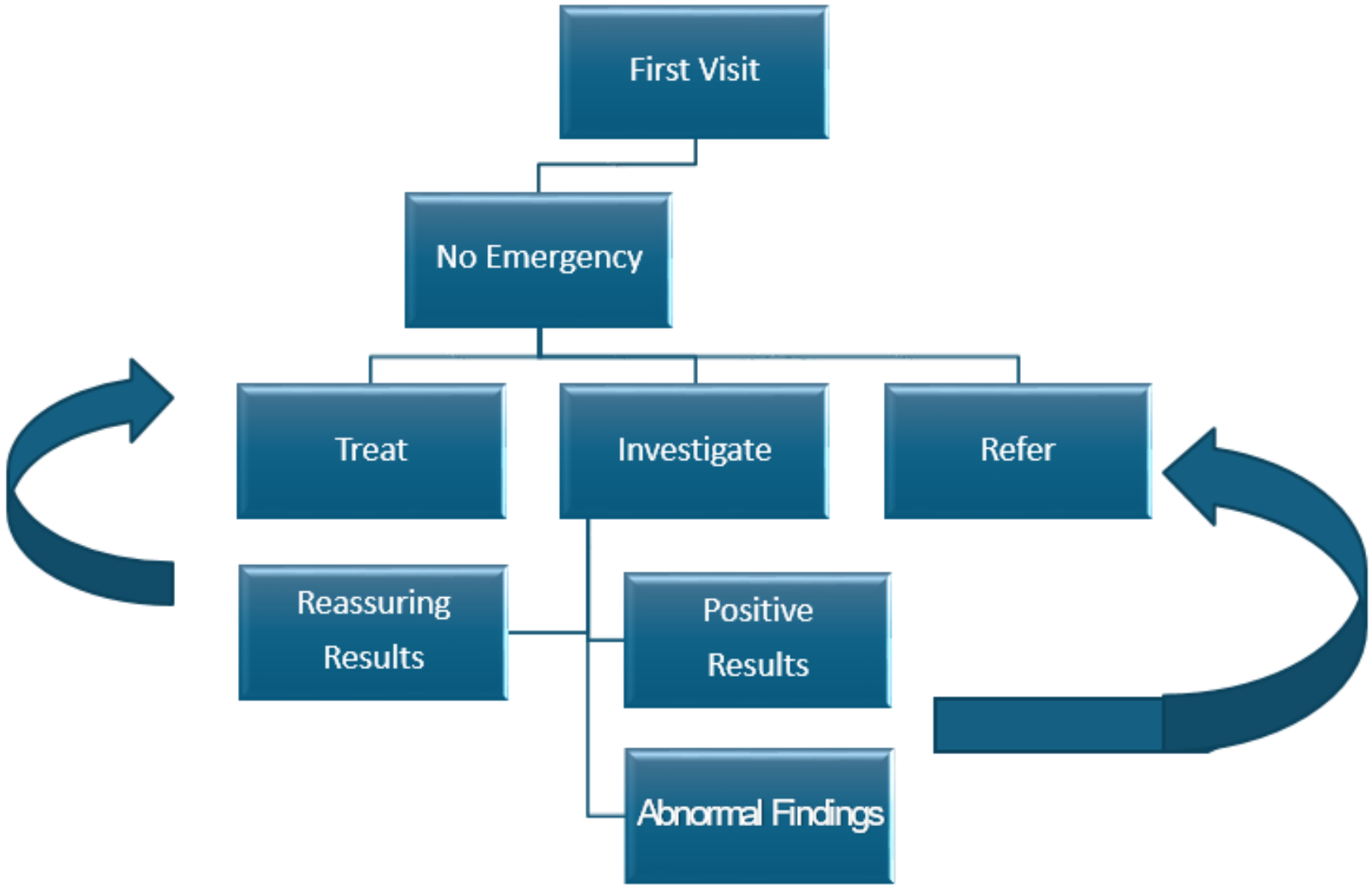
Reassuring results	Reassess symptoms and medicines. Consider functional diarrhea or IBS when criteria fit.
Positive celiac serology	Refer for diagnostic confirmation. Avoid a gluten-free diet before confirmation.
Persistent or abnormal findings	Refer for further evaluation, which may include colonoscopy and biopsies.

Calprotectin is not a cancer test. A normal result does not exclude every organic cause of diarrhea.

AGA chronic watery diarrhea guideline (2019)

NICE CG61: IBS in adults

Case 2: how the results change the pathway



STEP 3 • WHEN TO INVESTIGATE

Diarrhea case: targeted testing in primary care



CASE 2 • A 29-year-old with persistent watery diarrhea

8 weeks of watery stools, 4–5 times daily, with bloating.

No blood, weight loss, fever or nocturnal symptoms. Normal vitals and examination.

No recent antibiotics, high-risk travel or relevant family history.



1 WHY INVESTIGATE?

Chronic diarrhea: 4 weeks or longer
Persistent symptoms need a targeted work-up.

Review medicines, diet and exposures.

Assess hydration and symptom impact.

Do not assume IBS because the patient is young.
IBS requires a characteristic recurrent abdominal pain pattern and appropriate symptom duration.

IBS = irritable bowel syndrome

2 INITIAL TESTS

Blood



- CBC and chemistry as clinically indicated.
- Celiac screen: tTG-IgA plus total IgA.
- Use an appropriate IgG-based test if IgA deficient.

Stool



- Fecal calprotectin or lactoferrin.
- Giardia antigen or PCR.
- Add other infection tests according to exposures and clinical context.

Keep gluten in the diet during celiac evaluation.

tTG = tissue transglutaminase

3 ACT ON RESULTS

Positive infection test
Give targeted treatment and reassess.

Positive celiac serology
Refer for diagnostic confirmation.

Raised fecal inflammatory marker
Interpret in context; investigate or refer for possible inflammation.

Reassuring tests, symptoms persist
Reassess and consider referral for bile acid diarrhea or microscopic colitis.
Colonic biopsies may be needed even if the colon looks normal.

Normal calprotectin does not exclude every organic cause. It is not a cancer test.

ESCALATE IF THE PATIENT DETERIORATES

Blood, weight loss, anemia, nocturnal diarrhea or fever require reassessment.
Severe dehydration, hemodynamic instability or severe abdominal pain require urgent hospital assessment.

Illustrative case from the presentation. Sources: AGA chronic watery diarrhea guideline (2019); BSG chronic diarrhea guideline (2018).

Set a review date and track every result. Tests depend on the clinical context and local availability.

When to refer

Does this patient need a specialist, and how urgently?

Case 3: anemia presentation, referral and parallel care

Referral urgency



Emergency	Instability, severe acute bleeding, peritonitis, obstruction or inability to swallow saliva.
Urgent / expedited	Progressive dysphagia, suspected cancer, concerning anemia or probable active inflammatory disease.
Routine	Persistent symptoms despite appropriate care, diagnostic uncertainty or a need for specialist testing.

State the urgency and reason. Use the local pathway rather than a universal waiting-time target.

NICE NG12: suspected cancer referral

AGA iron-deficiency anemia guideline (2020)

NICE CG141: acute upper GI bleeding

Case 3: anemia with a bowel habit change



Is iron replacement alone enough?

A 62-year-old man reports fatigue and 2 months of altered bowel habit.

Hemoglobin 9.8 g/dL, MCV 72 fL, ferritin 8 ng/mL.

He is stable and has no visible rectal bleeding.

→ **The absence of visible blood does not remove the need for GI evaluation.**

AGA iron-deficiency anemia guideline (2020)

NICE NG12: suspected cancer referral

Case 3: expedited referral and parallel care



Refer

Use an expedited GI or suspected-cancer pathway for unexplained IDA with bowel change.

Investigate

Bidirectional endoscopy is usually indicated in men with unexplained IDA.

Treat and track

Start appropriate iron replacement. Monitor clinical status and ensure referral completion.

Improvement in hemoglobin does not establish the source of iron loss.

AGA iron-deficiency anemia guideline (2020)

NICE NG12: suspected cancer referral

A useful gastroenterology referral

How to Write a Report

The clinical question and requested urgency

Symptom timeline, weight trend, alarm features and relevant examination

Laboratory results with dates, imaging and previous endoscopy reports

Treatment tried, dose, duration, adherence and response

Family history, relevant medicines and anticoagulant use

Name who will follow outstanding results and confirm that the patient attends.
Suggested referral structure, not a mandated guideline format. Example: "Urgent assessment for unexplained iron-deficiency anemia with new bowel change. Stable 62-year-old man. Hb 9.8 g/dl, ferritin 8 ng/ml. Please assess for upper and lower GI sources." Include reliable patient contact details in the actual referral. Never include invented identifiers from a teaching case.

STEP 4 • WHEN TO REFER

Anemia case: specialist evaluation without delaying care



CASE 3 • A 62-year-old man with fatigue

2 months of altered bowel habit. Clinically stable.
No visible rectal bleeding.

Hemoglobin	MCV	Ferritin
9.8 g/dL	72 fL	8 ng/mL

Iron-deficiency anemia (IDA)

1 WHY REFER?

- Unexplained IDA in an adult man
- New change in bowel habit
- Possible occult gastrointestinal blood loss
- No visible bleeding does not exclude a GI source.
- Investigate the cause, including malignancy, ulcers and malabsorption.

Iron replacement alone is not sufficient.

2 HOW URGENTLY?

Expedited GI assessment
Use the local suspected-cancer pathway when indicated.

Likely investigation
Upper endoscopy plus colonoscopy to evaluate unexplained IDA.
Tests depend on symptoms, fitness and local practice.

Referral detaiis

- Symptom timeline and alarm features
- Dated CBC and iron studies
- Medicines, family history and prior investigations
- Treatment already started

CBC = complete blood count

3 CARE WHILE WAITING

- Start appropriate iron replacement.
- Review NSAID use and other potential bleeding risks.
- Monitor hemoglobin, symptoms and treatment tolerance.
- Consider celiac screening and other tests as indicated.
- Track the referral and confirm attendance.
- Assign responsibility for pending results.

Do not delay referral while waiting for the response to iron.

A rise in hemoglobin does not identify the cause or remove the need for investigation.

ESCALATE TO EMERGENCY ASSESSMENT

Active significant bleeding, syncope, hypotension, chest pain or severe breathlessness.
Urgency depends on clinical status, not the hemoiglobin value alone.

Illustrative case from the presentation. Sources: AGA GI evaluation of IDA (2020); BSG IDA guideline (2021).

Apply local referral pathways and individualize care.

Thank you for your attendance.

For more presentations, get the link below:
sarkiskhourysite.github.io/Medical-Presentations/viewer.html
Or scan the QR Code